

DISCHARGE MEDICATION RECONCILIATION & MEDICATION-SAFETY REVIEW

Mercy Vale Regional Medical Center — Hospital Medicine / Inpatient Pharmacy

Patient	Korvin Merrow
MRN / FIN	KM-6427819 / KM-2026-051877
DOB / Age / Sex	02/18/1964 · 62 y · Male
Unit / Room	5 West Medical / 5W-318
Allergy	Lisinopril — cough (ACE-inhibitor class intolerance). No known food/environmental allergies.
Admitted / Planned discharge	05/18/2026 (HD1) / 05/24/2026 AM
Attending / Service	Elian Vossmere, MD — Hospital Medicine
Document	Discharge medication reconciliation note, organized by disposition — for pharmacist + covering-hospitalist co-signature

Prepared in response to: Discharge Medication Reconciliation Request (Vossmere/Marlott, 05/24/2026) and Medication-Safety Handoff Addendum (Marlott/Vossmere, 05/24/2026).

Reconciliation basis: Verified 19-item home list (Initial Med Rec, Marlott PharmD, 05/18 — 18 of 19 items source-agreed across patient, family, pharmacy fill history, and PCP; prednisone the sole unverified item); inpatient MAR HD1–HD6; Cardiology (Caldrane), Nephrology (Solthar), Endocrinology (Veylorn) consult series; outpatient Rheumatology provenance (Halvek); pharmacy refill history (Harbor Crest); PT/OT, nursing, case-management/social-work, and family-conference documentation; renal/infection/hemodynamic trend summary.

COURSE IN BRIEF (context for the dispositions below)

62-year-old admitted with suspected urinary-source infection/sepsis physiology and AKI-on-CKD (stage 3 baseline), after ~3 weeks of decline, poor intake, evening confusion, and a near-fall. Multimorbid: HFrEF, CAD s/p remote mid-LAD DES (2018, single-agent antiplatelet — original DAPT long completed), HTN, hyperlipidemia, T2DM with neuropathy, OSA, PMR on chronic prednisone, anemia of CKD, steroid-related osteopenia, GERD, constipation tendency.

Six-day trajectory has been favorable and supports discharge planning: **Cr 2.62 – 1.80 mg/dL** (upper edge of the 1.6–1.8 baseline; AKI resolved by lab criteria), **K⁺ 5.1 – 4.4**, **WBC 15.6 – 9.4**, **afebrile since HD3**, **SBP 96 – 114–130**, intake 25–40% → 75–90%, at 97 kg dry weight. Residual **functional/cognitive and medication-management vulnerability persists** (Morse 65; reproducible OT medication-sorting errors; patient an unreliable narrator of his own regimen) — the safety plan below is built around that gap.

DISPOSITION SUMMARY

A. CONTINUED at discharge (pre-admission dose, documented outpatient indication)

Medication	Dose / route / freq	Indication	Inpatient course
Aspirin EC	81 mg PO daily	CAD secondary prevention (single-agent antiplatelet)	Given HD1–HD6
Atorvastatin	40 mg PO nightly	CAD / hyperlipidemia	Given HD1–HD6
Pantoprazole	40 mg PO daily	Chronic GERD	Given HD1–HD6
Calcium carbonate/vit D	600 mg/400 IU PO BID	Bone health w/ steroid exposure	Held HD1 (PO), then given
Ferrous sulfate	325 mg PO every other day	Anemia of CKD	Given on schedule (alternating-day)
Alendronate	70 mg PO weekly (Sun AM)	Steroid-related osteopenia	Not due inpatient; resume home weekly schedule
Nitroglycerin	0.4 mg SL PRN chest pain	CAD rescue	Available; never required
Acetaminophen	650 mg PO q6h PRN	Pain	PRN, sparing use
PEG 3350 / Senna	17 g daily PRN / 8.6 mg qHS PRN	Bowel regimen	PRN, intermittent
• Carvedilol 12.5 mg PO BID — CONTINUE (the only cardiorenal agent already restarted inpatient). Hold-and-call parameter: SBP < 100 mmHg or symptomatic lightheadedness/orthostasis. • Insulin glargine 18 units SC nightly — CONTINUE at the home dose (titrated 12–18 u inpatient, back to home dose by HD6). No new glycemic agent added in-hospital; none should be added at discharge. • Prednisone — CONTINUE without interruption (non-abrupt); reconciliation/handling in SC.			

B. HELD at discharge, WITH explicit restart parameters

All four agents below were appropriately held across the entire inpatient course for AKI/hemodynamic/volume vulnerability. Both Cardiology and Nephrology **explicitly declined to declare a unilateral or simultaneous restart safe at the moment of discharge.** The agreed position is a **staged, parameter-gated outpatient restart**, not a calendar restart and not all-at-once — “AKI resolved by labs on a monitored floor is not the same as restored home physiologic reserve” (Nephrology, HD6). **Do NOT auto-resume these on the discharge prescription.** Written restart instructions go to patient/family, PCP, Cardiology, and Nephrology.

Held medication	Home dose	Why held	Restart gate (meet BEFORE resuming)
Sacubitril/valsartan	24/26 mg PO BID	RAAS/neprilysin load on AKI; SBP margin	Restart at a cardiology/nephrology-coordinated outpatient visit once Cr stable at/near baseline and SBP reserve adequate; recheck BMP after restart
Spironolactone	25 mg PO daily	Hyperkalemia/AKI risk (K^+ 5.1 on admission)	Resume only with K^+ and Cr confirmed acceptable ; sequence after RAAS decisions; post-restart BMP
Empagliflozin	10 mg PO daily	Acute illness / poor intake / renal	Resume once euvolemic with stable intake and renal function at baseline
Furosemide	40 mg PO daily	Volume depletion / prerenal physiology	Resume only if volume reaccumulates (daily weight vs 97 kg dry weight, edema, dyspnea) — not reflexively

Recommended restart SEQUENCE (for the ambulatory teams; item #3 of the request). Staged, one agent at a time, each tied to the gate above and re-checked before the next step: 1. **Carvedilol — already done** (tolerated BID, HD4–HD6). 2. **Sacubitril/valsartan** next, at the **2-week Cardiology visit**, if SBP reserve allows — restart first because it carries the largest HFrEF mortality/readmission benefit and BP is the limiting variable. 3. **Empagliflozin** once euvolemic with stable intake (renal/cardiac benefit, modest BP effect). 4. **Spironolactone** last among the RAAS-axis agents, only after K^+ /Cr reconfirmed on the post-restart BMP. 5. **Furosemide — conditional, symptom/weight-driven**, not scheduled; this patient was volume-depleted, not congested, throughout.

Rationale (one line): Cardiology pressed to preserve GDMT/sequence early; Nephrology held reserve as fragile and restart as staged/lab-gated — complementary, not contradictory. Carvedilol captured early; the renally/hemodynamically sensitive agents are staged outpatient against hard parameters, with final agent/dose/timing the ambulatory decision of Cardiology + Nephrology written against the gate value on the day the order is placed.

C. DOSE-MODIFIED / RECONCILED-BUT-UNVERIFIED

Prednisone (PMR) — CONTINUE a non-abrupt daily dose at discharge; final taper is an OUTPATIENT rheumatology decision. No numeric long-term outpatient dose is asserted from inpatient sources.

Reconciliation across all available sources: - **Rheumatology (Halvek, authoritative for intent)**: several-year PMR course; most recent documented intent is a **cautious slow taper while controlled — NOT cessation; avoid abrupt discontinuation**. Deliberately states **no numeric dose**, and defers “what was most recently written” to pharmacy fill history / active e-prescription. - **Pharmacy fill history (operative for what was written)**: a downward trajectory over 6 months — **10 mg** (11/2025, 12/2025, 01/2026) — **5 mg** (from 02/2026), with directions varying across fills (“1 tab daily,” “...then decrease per schedule,” “taper per provider instructions,” and a “½ tablet daily” = **2.5 mg** on 03/22). **Most recent dispense 05/06/2026 = 5 mg tablet, taper-style directions**. - **Patient**: “still on a little... might have been tapering off” — cannot state strength or schedule; unreliable. - **Family (Mara)**: taper understood; bottle labels and actual intake may not match; multiple instruction sets seen; cannot reconstruct recent day-by-day adherence. - **PCP (Quenor)**: defers to rheumatology; PCP list reflects historical instructions and is NOT the current taper directive. - **Endocrinology (Veylorn)**: continue non-abrupt (stress-equivalent) coverage during illness; **do not invent a new long-term dose inpatient**; adrenal insufficiency **NOT established** but chronic exposure is a real risk substrate; finalize a structured taper with Halvek after the acute illness resolves.

Reconciled conclusion: multiple strengths/directions have been in circulation and **no source can verify what was actually taken at home** — a precise pre-admission home dose is unrecoverable and must not be fabricated. Most-recent *written* strength is **5 mg with taper intent**. **Disposition**: discharge on a single, unambiguous **continuation dose consistent with the most recent 5 mg dispense, taken once daily without interruption**, explicitly labeled as a bridge pending rheumatology, with a **rheumatology appointment within 2–4 weeks (coordinated with Endocrinology) to set the definitive taper**. Do not discontinue; do not hand the patient overlapping older bottles (10 mg; “½ tablet”). Sick-day/stress-dose teaching per SE.

D. INPATIENT-SPECIFIC — do NOT carry forward (request item #4; handoff item #3/#6)

Each item below was ordered for an inpatient-only indication and has **no independent outpatient justification**; none should appear on the discharge prescription.

- **Correctional insulin lispro (sliding scale)**: inpatient glycemic coverage during acute illness; tapered to minimal use by HD6. **Not a home order** — home diabetes control is basal glargine ± the held oral agents, reviewed outpatient.
- **IV crystalloid (0.9% NS) cautious-volume strategy**: off by HD4 (saline lock). Inpatient only.
- **Empiric antibiotics** — see antibiotic disposition below.

Antibiotic disposition (request/handoff item #6): Empiric ceftriaxone 1 g IV q24h HD1–HD3, stepped down to cefpodoxime 200 mg PO BID from HD4, with clinical resolution (afebrile since HD3, WBC 9.4, symptoms resolved). No final urine-culture speciation/sensitivity is documented in the record.

Recommendation — option (b): complete a short, defined ORAL course with a named stop date. Continue cefpodoxime 200 mg PO BID to complete a 7-day oral course counting from the HD4 (05/21) switch – STOP after the evening dose on 05/27/2026 (i.e., ~3–4 additional days post-discharge). Rationale: treating urinary-source sepsis physiology (not simple cystitis) in a diabetic CKD patient favors completing a defined total course over same-day cessation; a discrete stop date prevents an open-ended outpatient antibiotic. If a resulted culture/sensitivity is located before discharge, reconcile agent/duration to it and amend this instruction. This is the one inpatient-initiated agent that is *intended* to carry forward briefly — with an explicit stop date so it does not become a chronic medication.

HOME MEDICATION-MANAGEMENT SAFETY PLAN (request item #5; handoff item #4)

This regimen is complex (~18 active scheduled/PRN items plus prednisone), layered with **new held-and-not-restarted** agents and a **changed prednisone handling** instruction, in a patient with documented, reproducible medication-management deficits and high fall risk.

Documented risk substrate: OT found the patient **could not reliably organize his 19-item regimen** — errors clustered at **BID timing** and **PRN-vs-scheduled** distinction, reproducible across sessions, and **he does not self-detect errors in real time**; he could not state recent dose changes or the prednisone taper. Nursing repeatedly recorded “I think so” when he was asked whether a dose was taken. PT: **Morse 65, high fall risk, orthostatic vulnerability, rolling walker** — directly relevant given the hemodynamically active held/continued agents (carvedilol, the held diuretic/RAAS agents) and **gabapentin** sedation/fall-risk. Baseline cognition is only mild forgetfulness, so this is a true deviation. Caregiver: wife Mara (proxy) engaged but capacity-limited; daughter Lenora covers weekend days/transport but **not weekday mornings** — a named supervision gap.

Plan: 1. **Pre-filled weekly pill organizer, filled by the caregiver (not the patient alone)** for at least the first weeks (OT). Discard/sequester older prednisone bottles (10 mg; “½ tablet”) so only the single current instruction is in the home. 2. **Family-supervised administration with daily check-in at medication times for the first 5–7 days, verifying the dose was actually taken** (OT) — overnight + AM + PM coverage explicitly assigned among Mara/Lenora, with the weekday-morning gap named and solved before discharge. 3. **Home health nursing for early post-discharge medication review** (Case Management — Keystone HomeCare candidate); confirm acceptance before discharge. Single accountable owner for the teach-back session to be named. 4. **One consolidated written medication list** distinguishing CONTINUED / HELD-with-restart-parameters / STOPPED (inpatient-only) / PRN, with the antibiotic stop date and the prednisone bridge explicitly stated. Avoid the patient bearing sole responsibility for any new/changed medication during the early window (OT). 5.

Environment/device: rolling walker (not intermittent cane), bedside commode initially given evening confusion + nocturia, bathroom grab bars, remove loose rugs, supervised mobility 5–7 days (PT). 6. **NSAID avoidance** at home and renal-dose review of any new prescription (Nephrology). 7. **Pharmacy logistics:** Harbor Crest Community Pharmacy (412) 555-0490 — coordinate first-fill timing for the prednisone continuation and the short antibiotic course; confirm no duplicate/obsolete prednisone fills are dispensed.

POST-DISCHARGE RENAL & CARDIAC MONITORING PLAN (handoff item #5)

The trend summary flags that **no scheduled post-discharge monitoring plan exists**; one is specified here.

- **Labs: BMP (creatinine/eGFR and potassium) within 3–5 days of discharge**, at Mercy Vale Outpatient Laboratory — Harbor Crest, (412) 555-0455. Repeat **BMP within 48–72 h after ANY cardiorenal agent is restarted**. Check hemoglobin at PCP visit (anemia of CKD).
 - **Home weights:** daily AM weight against the **97 kg dry-weight baseline**; contact clinician for **gain > ~1.5–2 kg in 2 days** or new edema/orthopnea/exertional dyspnea (HFrEF decompensation).
 - **BP/HR parameters:** recheck orthostatics; **call for SBP < 100 mmHg, symptomatic lightheadedness, or near-fall** (carvedilol hold-and-call trigger) — equally important because the four BP-active agents remain held.
 - **Accountable receivers:** BMP and weights routed to **PCP (Quenor, within 7 days)**; restart-titration labs/BP to **Cardiology (Caldrane, within 2 weeks)** and **Nephrology (Solthar, within 1–2 weeks; earlier if any restart is initiated)**. Glycemic/steroid review with **Endocrinology (2–4 wks)** and **Rheumatology (Halvek, 2–4 wks)**. **Transportation and outpatient-lab logistics are unconfirmed per Case Management and must be closed before discharge** or the monitoring plan fails.
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TEACH-BACK RECOMMENDATIONS — patient + primary caregiver (request item #6)

Conduct with Mara present (Lenora by phone), teach-back confirmation at the session **and again at ~24 h**; map directly to the family's five recorded pre-discharge questions.

1. **Changed items:** carvedilol is back on; glargine is back to the usual 18 units. Organize by AM / PM / nightly / weekly / PRN; have the caregiver teach **back the BID vs PRN distinction** (the specific OT-identified failure point).
2. **Held-and-not-yet-restarted items:** sacubitril/valsartan, spironolactone, empagliflozin, and furosemide are **deliberately paused** and will be **restarted one at a time by the heart/kidney doctors after lab checks** — “do not restart these on your own.” Explain heart and kidney meds **together**, as Mara requested.

3. **Prednisone:** take the single current daily dose **every day without stopping**; use only the one current bottle (remove the older/"½-tablet" bottles); the **rheumatologist will set the taper** — do not change or stop it on your own.
 4. **Glycemic regimen:** 18 units glargine nightly; the inpatient mealtime sliding-scale insulin is **stopped**; bring the glucose log to PCP/Endo.
 5. **Prednisone sick-day/stress rule:** with fever, persistent vomiting, or significant dehydration, **do not stop the steroid** — call the physician promptly for stress-dose guidance (Endocrinology).
 6. **Warning signs – who to call:** return/call for fever, worsening weakness or new confusion, recurrent lightheadedness or near-fall, chest pain, shortness of breath, weight gain/edema, or decreased intake. Routine – PCP/specialty clinic; urgent/red-flag – ED or (412) 555-0150. Confirm the caregiver can state at least three red flags back.
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OPEN ITEMS TO CLOSE BEFORE THE DISCHARGE ORDER IS WRITTEN

- Final cardiorenal restart instructions written against the day-of parameter values (none restarted as of HD6 18:00 except carvedilol).
 - Prednisone continuation prescription issued as a single unambiguous dose; obsolete bottles reconciled; rheumatology follow-up booked.
 - Antibiotic stop date confirmed (and reconciled to culture data if resulted).
 - Home health acceptance, DME delivery (walker/commode/grab bars), transportation, and outpatient-lab logistics confirmed (Case Management).
 - Named owner for the family teach-back session; supervision windows (incl. weekday AM gap) assigned.
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CO-SIGNATURE

This reconciliation note is submitted for co-signature and release with the discharge paperwork. Page Pharmacy (412) 555-0170 or Hospital Medicine (412) 555-0301 with questions.

Reviewing Pharmacist: _____ Date/Time: _____

Covering Hospitalist: _____ Date/Time: _____